Week 7 Project Summary

1. What I planned to test

My plan was to check how accurate and easy to use my Week 6 Power BI dashboard was. I wanted to test the KPIs, charts, filters and main findings using the same HealthConnect Rwanda database from Week 6.

2. What I actually tested

I went back to the original CSV files and recalculated the main KPIs to see if they matched the dashboard figures. I also checked the charts, rankings, blank categories and values that were not displaying properly.

I tested the slicers, drill-through, tooltips and bookmarks to make sure they worked as expected. I also checked some of the main findings across different insurers, months and hospitals.

3. Most important testing results

The KPIs I checked matched the original data. These included RF9.31 billion in revenue, RF8.39 billion collected, 50% claim approval and an average satisfaction score of 2.99.

The first round of testing revealed seven issues. I made corrections and tested the affected areas again. All the dashboard checks passed after the fixes. The cross-track test is still outstanding.

4. Issues or weaknesses identified

I noticed that the facility matrix and bed chart were showing the same figures for every hospital instead of displaying each hospital's actual numbers.

The department ranking was also arranged alphabetically rather than by satisfaction score. Some cards and charts were cutting off important values. During retesting, I also found a blank row showing 100% bed utilisation and some cards displaying filtered figures in a way that made them difficult to read.

5. Improvements and refinements made

I corrected the facility field used in the matrix and bed chart so that each hospital would show its own figures. I also adjusted the bed utilisation measure, sorted the department ranking correctly and removed blank categories.

Other changes included making the revenue card and monthly trend chart wider, removing an incorrect total row from the growth table and adjusting the card display units so that the figures could be read properly.

6. Retesting results

After making the changes, I tested the affected areas again and confirmed that they were displaying the expected results.

For example, HF001 now shows 273 appointments, 64 staff, 100 beds and 10 equipment. When HF011 is selected, the dashboard correctly displays 273 appointments and RF44.21M in revenue.

7. Track(s) I collaborated with

I have not completed any collaboration with another HC-POD track yet because I could not get a teammate available at the time of writing.

8. What was tested collaboratively

No collaborative testing has taken place yet. However, I have prepared the findings from my analysis for the Project Management and Data Science teams.

The plan is to share these findings with them, get their feedback, allow them to point out anything I may have missed and retest any issues they identify.

9. What changed as a result

The dashboard is now easier to read and use. The corrections also helped me separate findings that are supported by the data from those that need further investigation.

I also updated my recommendations based on what I discovered during testing.

10. Key findings and validation outcomes

The claim approval rate remained between 49.3% and 50.9% across all insurers. Emergency appointments also stayed between 30.1% and 36.5% every month. This shows that both findings are consistent across the data.

However, I found that bill status does not always agree with actual payments, some written patient comments do not match their ratings, and hospital satisfaction scores are based on a relatively small number of feedback records, between 72 and 102 per hospital.

11. Major challenges encountered

One challenge was that the Week 7 brief focused on attendance and no-shows, but my Week 6 dataset did not contain an attendance field. This meant I could not properly analyse those areas.

I also spent time tracing some dashboard display problems to the wrong fields and card display settings. Another challenge was not having a teammate available for the cross-track testing.

12. Important decisions made

I decided to continue using the Week 6 dataset, as required by the brief.

I also used actual payment amounts instead of bill status when analysing collections, and star ratings instead of written comments when checking patient satisfaction.

I treated the HF014 satisfaction result as something worth investigating rather than a definite conclusion because the feedback sample was small. I also used month-on-month growth instead of year-on-year growth because the available data only covers 2026.

13. Remaining limitations

There are still some gaps in the dataset. Only 5,000 out of 60,000 bills and diagnoses could be linked to appointments. Hospitals are identified by codes instead of names, and all 2,000 beds are recorded as available.

The dataset also does not include laboratory, prescription, admission, referral or attendance information. The currency used in the analysis is assumed to be Rwandan francs.

14. Remaining issues or dependencies

The cross-track test still needs to be completed. I also identified 821 appointments with future dates that need to be checked against the original records.

Bill status also needs to be reviewed and matched with actual payment records.

15. My contribution to the overall HealthConnect project

My contribution was to check and validate the data analysis behind the HealthConnect dashboard. This helped confirm which figures could be relied on and which findings need further investigation.

It also gives the other teams a clearer picture of the data issues that may affect their own work.

16. What must be completed before Week 8

Before Week 8, I need to complete a cross-track review with someone from Project Management or Data Science and include their feedback in my report.

I also need to share my validated findings with the relevant teams and prepare the final presentation using the results from my testing.